ICOPE

INTEGRATED CARE FOR OLDER PEOPLE

IMPLEMENTING CARE FOR HEALTHY AGEING

Facilitator Guide: Module 03

WHO / Blink media - Uma Bista

World Health Organization

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Introduction to the Guide

Welcome to the Facilitator Guide for the WHO Integrated Care for Older People (ICOPE). This guide serves as a roadmap for the facilitators, helping them navigate through the session while ensuring that key topics are covered and participants are engaged. It may also include tips, potential challenges and suggested ways to handle different situations that may arise during the session.

Iconography

The following icons are used in the Facilitator Guide to indicate the type of content being presented.

Icon	Action	Description
	Session Title	Indicates the name of the session being conducted.
	Session Objectives	Lists the learning objectives to be achieved.
	Timing	Indicates the duration of the session or activity.
	Show	Indicates the slide to be presented.
	Say	What to say or explain while facilitating. It will contain the recommended script/ answers to be discussed.
	Ask	Ask the participants a question and encourage them to respond.
	Do	What to do to facilitate an activity or provide guidance to learners.
	Play	Indicates a video clip to be presented.

Session Structure

This facilitator guide is organised according to the way you will present the material on each slide:

- **Show** – The slides
- **Say** – This is a scripted narrative outline for you.
- **Ask** – Questions to prompt dialogue with and among the participants
    - The dialogue associated with the questions should take between 5 to 10 minutes. However, you will need to use your best judgement about the time to dedicate to the question-and-answer sessions. Some sessions may last longer.
- **Do** – Prompts you to do an action

Keep in mind that this Facilitator Guide is only a roadmap. You are expected to apply your own voice and experience in making this tool work for you. The 'Say' sections are simply indications; you can use them as a script when you feel the need to, but you can and should adapt it to suit your natural training style.

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Add your own personal touch and personality to every training, while being careful to stick to the session objectives.

A key component of successful face-to-face training is establishing trust and rapport with your learners. Use your own good judgment to assess the attitude and cultural sensitivities of the people in your workshop. Adapt your training techniques and approach accordingly.

You are going to be great at conducting this training.

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Module 3: Implementing Care for Healthy Ageing

	Session Title:	**Implementing Care for Healthy Ageing**
	Timing:	10 min
	Session Objectives:	• Understand the concept of Universal Health Coverage. • Explain the need for integrated care of older people. • Describe the ICOPE Implementation Framework. • Explain the domains of Functional Ability and Intrinsic Capacity. • List the principles of ICOPE person-centred care. • List the available resources to support the integration of care for older people. • Describe the evidence-based recommendations to prevent and manage impairments in intrinsic capacity.

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Implementing Care for Healthy Ageing

Time: 10 min

Do:

- *Formal welcome*
- *Introduction of facilitator*

Show: Slide 1

Say:

Welcome to the module on Implementing Care for Healthy Ageing. In this session, we will cover the following.

- Understanding the concept of Universal Health Coverage.
- Explaining the need for integrated care of older people.
- Describing the ICOPE Implementation Framework.
- Explaining the domains of Functional Ability and Intrinsic Capacity.
- Listing the principles of ICOPE person-centred care.
- Listing the available resources to support the integration of care for older people.
- Describing the evidence-based recommendations to prevent and manage impairments in intrinsic capacity.

We'll delve into each of these topics to gain a comprehensive understanding of implementing care for healthy ageing. Let's get started.

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Learning Objectives

Show: Slide 2

Say:

By the end of this module, you'll have a solid grasp of essential components for effective care of older adults. We'll cover:

1. **Universal Health Coverage:** You'll understand how this concept ensures equitable access to healthcare for all, including older people.
2. **Integrated Care:** We'll explore why it's crucial to provide holistic and coordinated care for the ageing population.
3. **ICOPE Implementation Framework:** You'll learn about a structured approach to delivering integrated care, focusing on Functional Ability and Intrinsic Capacity.
4. **Functional Ability and Intrinsic Capacity:** We'll delve into these domains, highlighting their significance in promoting healthy ageing.
5. **ICOPE Person-Centred Care Principles:** You'll discover how to tailor care approaches to meet the unique needs and preferences of older adults.
6. **Available Resources:** We'll list resources supporting the implementation of integrated care, empowering you with practical tools.
7. **Evidence-Based Recommendations:** Lastly, we'll discuss strategies to prevent and manage impairments in intrinsic capacity, ensuring optimal health outcomes for older people.

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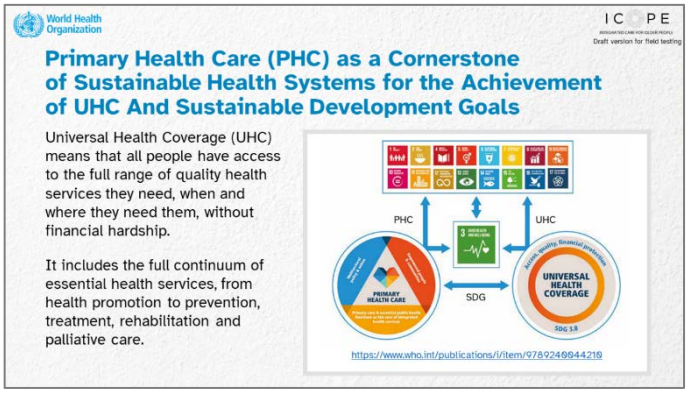
Primary Health Care (PHC) and Sustainable Health Systems

Show: Slide 3



Say:

In this slide, we will discuss the importance of Primary Health Care (PHC) in achieving Universal Health Coverage (UHC) and the Sustainable Development Goals (SDGs).

UHC ensures everyone can access quality health services without financial hardship. PHC is vital for this, offering a wide range of essential services from prevention to palliative care.

PHC's vision involves three key elements:

1. Integrated health services, focusing on primary care and public health.
2. Multisectoral policies and actions.
3. Empowered communities and people.

Together, these elements work towards the achievement of SDGs, in particular the third one focused on good health and wellbeing.

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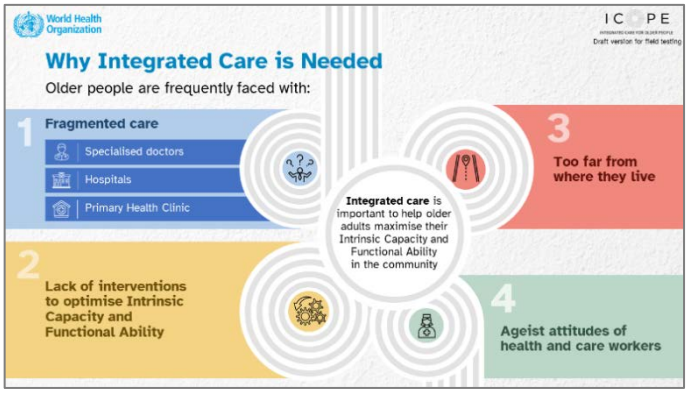
Why Integrated Care is Needed

Show: Slide 4



Ask:

Why integrated care is needed?

Say:

Let's discuss why integrated care is crucial for older adults:

1. **Fragmented Care:** Older people often receive care from various specialised doctors and hospitals. The lack of adequate connection and communication among them frequently results into fragmentation of care delivery.
2. **Lack of Interventions:** There's a shortage of interventions aimed at optimising Intrinsic Capacity and Functional Ability for older adults. The usual focus on “disease” prevents the prioritisation of more meaningful and comprehensive activities fostering capacities and abilities.
3. **Accessibility Issues:** Many older people face challenges accessing care due to geographical distance from care facilities. At the same time, please consider that access to care may be particularly challenging for persons with physical, cognitive, and/or social issues.
4. **Ageist Attitudes:** Negative attitudes toward ageing from health and care workers can hinder older adults' access to quality care. Integration of care implies a multidisciplinary and multisectoral approach, promoting a more inclusive approach to the priorities of the older person.

Integrated care plays a vital role in addressing these challenges by ensuring that older adults receive comprehensive and coordinated care aimed at maximising their Intrinsic Capacity and Functional Ability within the community. Let's explore the benefits of integrated care in enhancing the well-being of older people.

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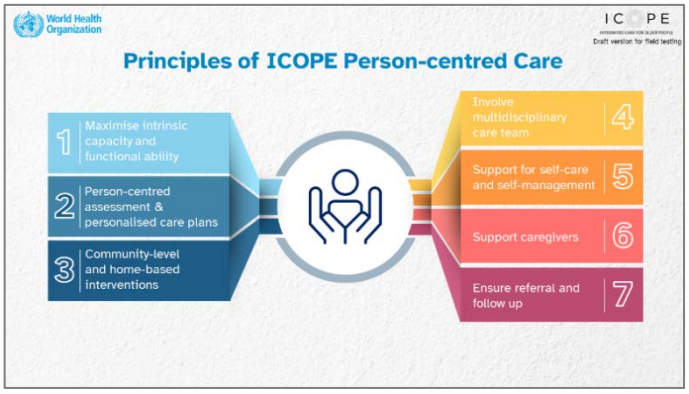
Principles of ICOPE Person-centred Care

Show: Slide 5



Say:

Let's explore the principles of ICOPE person-centred care, where ICOPE stand for Integrated Care for Older People:

1. **Maximise intrinsic capacity and functional ability:** The aim is not, as frequently occurring, the treatment of "the disease" per se, but the optimisation of intrinsic capacity and functional ability.
2. **Person-centred assessment and personalised care plans:** It's essential to involve the older person in decision making and goal setting, prioritising goals based on their values, needs and preferences. Only through such a comprehensive approach, it will be possible to develop and implement a person-centred care plan.
3. **Community-level and home-based interventions:** Opportunities to engage communities in supporting care must be explored to guarantee acceptability, adherence, and sustainability. Multicomponent interventions should be designed to deliver care at home and in the community as much as possible to meet the person's preferences and values.
4. **Involve multidisciplinary care team:** Assessment and management should involve a multidisciplinary team of health and social care workers. This implies the development of a professional environment supporting teamwork and collaboration.
5. **Support for self-care and self-management:** Support for self-care and self-management includes providing older people with information, skills and tools, respecting their autonomy and abilities to direct their own care.
6. **Support caregivers:** It is important to provide essential information about the older person's health conditions, training to develop practical skills, and exploring opportunities across sectors in support of caregivers (also involving communities and neighbourhoods).
7. **Ensure referral and follow up:** Strong referral pathways to specialised care, such as geriatric care, acute care in case of unforeseen events, and palliative care, are crucial. Furthermore, regular follow-up is essential for early detection of changes in functional status.

These principles guide us in providing holistic and person-centred care to older people, ensuring their well-being and quality of life. Let's explore how we can implement these principles effectively in our practice.

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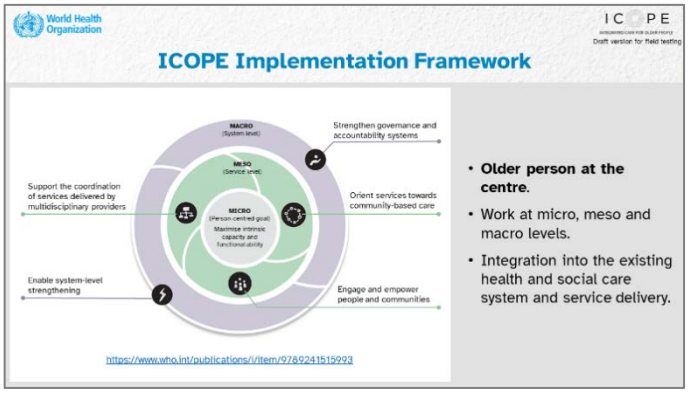
ICOPE Implementation Framework

Show: Slide 6



Say:

As we explore the ICOPE implementation framework, it's evident that placing the older person at the centre is paramount. We must work across all levels of the framework – Micro, Meso and Macro – to effectively integrate the ICOPE approach into existing health and social care systems.

At the Macro level, our focus is on system-level strengthening and governance to enable seamless coordination.

Moving to the Meso level, we aim to support multidisciplinary coordination, orient services towards community-based care and empower both individuals and communities.

Finally, at the Micro level, our person-centred goal is to maximise intrinsic capacity and functional ability.

By simultaneously working across these levels and harmonising efforts, we can ensure holistic and effective care delivery for older adults.

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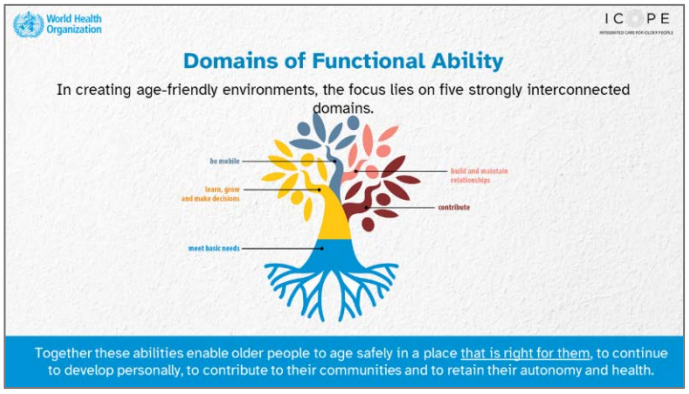
Domains of Functional Ability

Show: Slide 7



Say:

Let's explore the essential domains constituting functional ability, and stemming from intrinsic capacity, environment and the interactions between the two. There are five key domains of functional ability that are deeply intertwined:

Meeting Basic Needs: This includes essentials like housing, financial and personal security.

Learning, Growth and Decision-Making: Encouraging continuous personal development and decision-making abilities.

Mobility: Ensuring older people can move freely and independently within their environment.

Building and Maintaining Relationships: Facilitating social connections and nurturing meaningful relationships.

Contribution: Empowering older adults to actively contribute to their communities.

Together, these abilities enable older people to age safely in a place that is right for them, to continue personal development, to contribute to their communities and to retain their autonomy and health.

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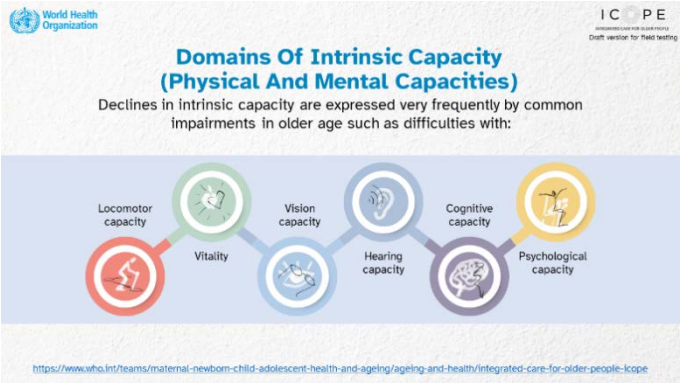
Domains of Intrinsic Capacity (Physical and Mental Capacities)

Show: Slide 8



Say:

Although critical for the individual's healthy ageing, impairments in intrinsic capacity, including hearing loss, vision issues or mobility limitations, are often overlooked or dismissed as part of "normal ageing" by health and care workers. Early signs of decline, such as slower gait or muscle weakness, frequently go unnoticed and untreated, losing clear opportunities to intervene when the problem is still at its early stages and potentially more reversible. Moreover, this lack of recognition and management can lead to older people disengaging from services and caregivers feeling unsupported. We urgently need comprehensive community-based approaches and interventions to prevent declines in capacity and provide vital support to caregivers.

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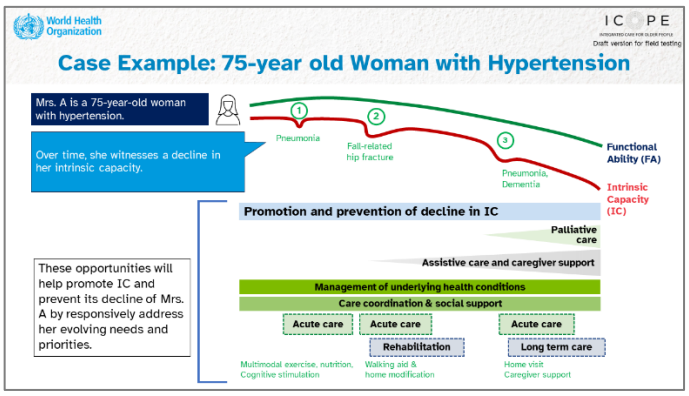
Case Example: 75-year old Woman with Hypertension

Show: Slide 9



Say:

Now let's look at a scenario and break it down to understand it. The green line represents the trajectory of functional ability. It tends to remains relatively stable over time as its domains represent abilities that consistently define meaningful goals across older life. The red line instead represents intrinsic capacity. As evident, it is not so linear as affected by changes due to the ageing process and incident health-related events. The gap between the red and green lines represents the environment, more or less supportive in the full expression of intrinsic capacity into functional ability.

At the bottom of the slide, you can see the many care services and activities that are or may become available according to the evolving needs of the older person. For example, in the case example depicted in the slide, Mrs. A, a 75-year-old woman witnesses a progressive decline in her intrinsic capacity. She initially suffers from an acute illness without major consequences (#1, e.g., a pneumonia followed by full recovery), then an acute condition affecting her intrinsic capacity (#2, e.g., a hip fracture causing mobility limitations and early cognitive impairment), and finally an acute event in the presence of a significant loss of intrinsic capacity (#3, e.g., delirium in a person with pneumonia and dementia). Different care opportunities are activated over time to address the emerging issues, as ad hoc temporary solutions (e.g., hospitalization, rehabilitation) or on a continuous basis (e.g., assistive care and caregiver support, palliative care). The example shows how the system may have the resources to address the person's priorities, but they need to be integrated and coordinated to ensure the necessary continuum of care.

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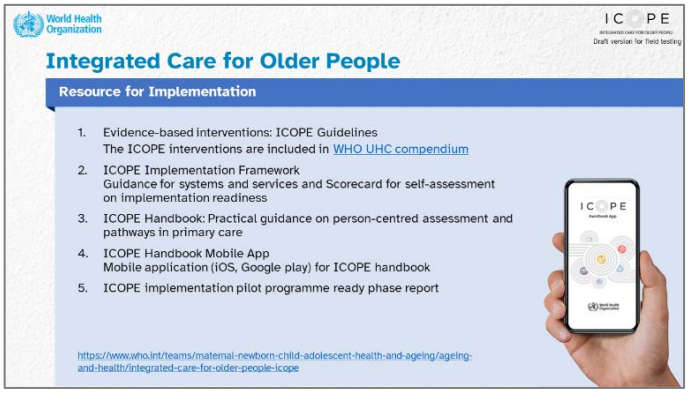
Integrated Care for Older People

Show: Slide 10



Say:

Let's explore the resources available to facilitate the implementation of integrated care for older people:

1. **Evidence-based interventions:** The ICOPE Guidelines, which are included in the WHO UHC compendium, offer a comprehensive set of interventions that can be put in place at the primary health care level to improve intrinsic capacity in community-dwelling older persons.
2. **ICOPE Implementation Framework:** This framework provides guidance for systems and services in the assessment of the implementation readiness. The document includes Scorecards to make objective and structured the evaluation.
3. **ICOPE Handbook:** A practical guide on person-centred assessment and pathways in primary care, offering valuable insights to health and care workers on how to screen, assess, and manage the intrinsic capacity impairments of the older person.
4. **ICOPE Handbook Mobile App:** Access the ICOPE handbook conveniently through the mobile application available on iOS and Google Play.
5. **ICOPE implementation pilot programme ready phase report:** This document reports and discusses the experiences of pilot studies conducted in different countries and regions that have been conducted the implementation of the ICOPE approach.

These resources are invaluable for implementing ICOPE effectively and promoting healthy ageing in our communities.

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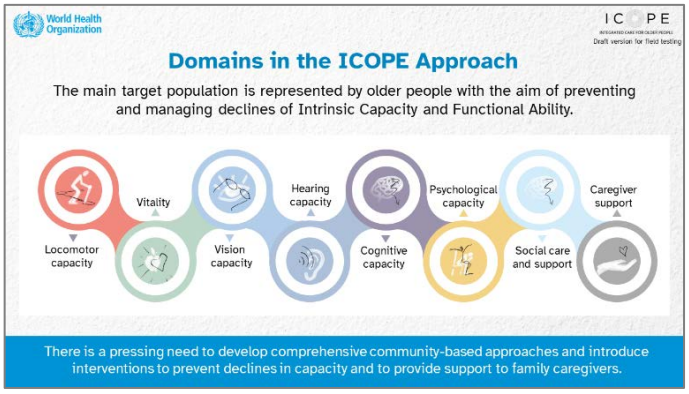
Domains in the ICOPE Approach

Show: Slide 11



Say:

Let's delve into the domains covered by the ICOPE Approach:

1. Locomotor Capacity
2. Vitality
3. Vision Capacity
4. Hearing Capacity
5. Cognitive Capacity
6. Psychological Capacity
7. Social Care and Support
8. Caregiver Support

As you can see, we have the six domains constituting Intrinsic Capacity, and two domains capturing critical aspects of the environment surrounding the older person: the availability of social care and support, and the caregiver support. The evaluation of these eight domains is fundamental to define priorities and consistently develop person-centred interventions aimed at preventing and managing declines in Intrinsic Capacity and Functional Ability.

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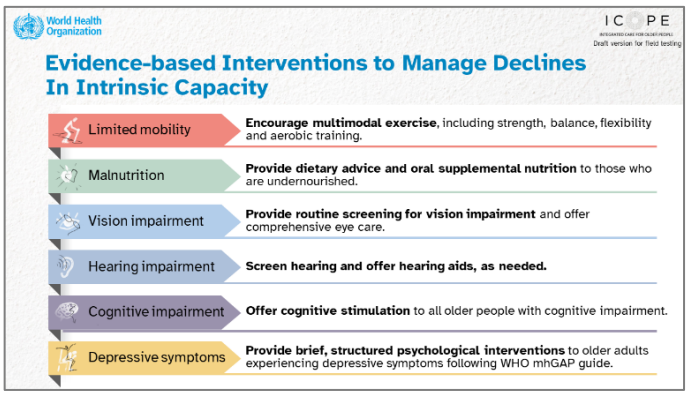
Evidence-based Interventions to Manage Declines in Intrinsic Capacity

Show: Slide 12



Say:

Let's explore evidence-based interventions to manage declines in intrinsic capacity. These interventions, detailed in the ICOPE Guidelines, are providing immediate opportunities to act in case of impairment in the domains of intrinsic capacity. In particular, in case of…

- **Limited mobility:** It is recommended to encourage multimodal exercise, incorporating strength, balance, flexibility and aerobic training to improve mobility.
- **Malnutrition:** Provide dietary advice and oral supplemental nutrition for older people who are undernourished.
- **Vision impairment:** Conduct routine screening for vision impairment and offer comprehensive eye care.
- **Hearing impairment:** Screen for hearing loss and provide hearing aids as necessary.
- **Cognitive impairment:** Offer cognitive stimulation to older adults with cognitive impairment.
- **Depressive symptoms:** Provide brief, structured psychological interventions based on the WHO mhGAP guide.

These interventions are grounded in evidence and aim to effectively manage declines in intrinsic capacity, promoting overall well-being among older people.

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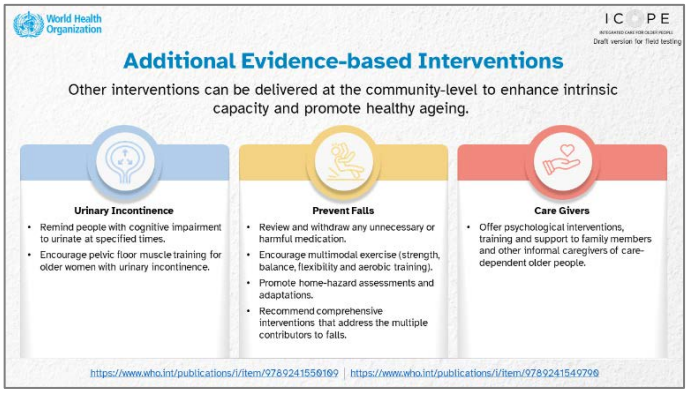
Additional Evidence-based Interventions

Show: Slide 13



Say:

The ICOPE Guidelines also include additional evidence-based interventions that are critical to consider for addressing major priorities for older people:

Urinary Incontinence:

- Remind older people with cognitive impairment to urinate at specified times.
- Encourage pelvic floor muscle training for older women experiencing urinary incontinence.

Fall Prevention:

- Review and discontinue any unnecessary or harmful medications.
- Promote multimodal exercise, including strength, balance, flexibility and aerobic training.
- Advocate for home-hazard assessments and adaptations to reduce fall risks.
- Recommend comprehensive interventions addressing multiple factors contributing to falls.

Caregiver Support:

- Provide psychological interventions, training and support to family members and informal caregivers of care-dependent older people.

These evidence-based interventions are essential in addressing specific challenges faced by older adults and their caregivers, with the ultimate goal of promoting overall well-being and quality of life.

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Summary

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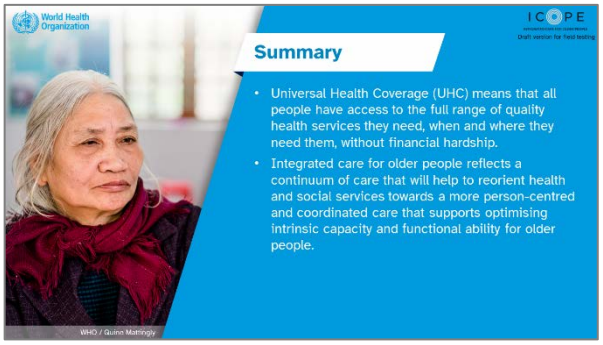

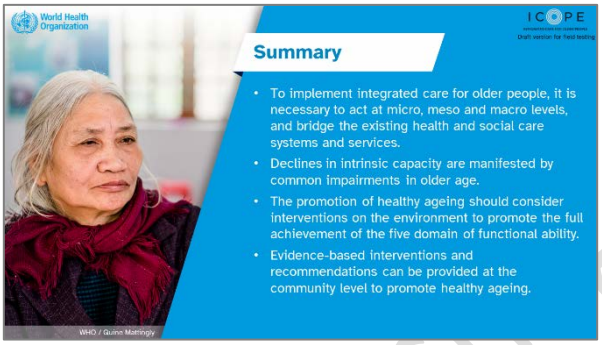

Do:

Go through the slides and recap the points discussed during the session.

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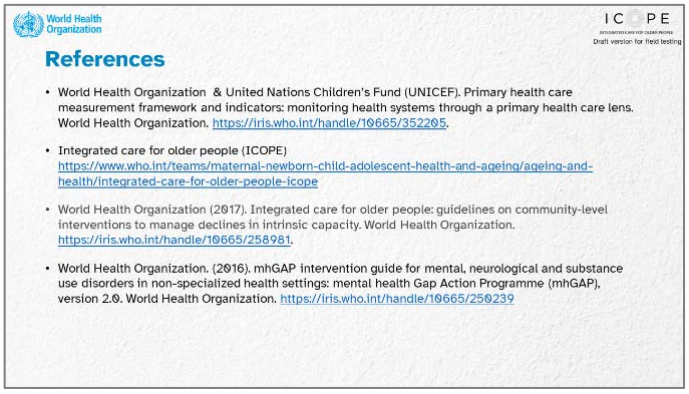
References

Show: Slide 16



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